

The Global Burden of Mental Illness: Prevalence, Impact, and the Treatment Gap

A data-driven overview of global mental health conditions, their disease burden, and the persistent gap between need and treatment (global data, 2023).

Audience: Public health policy analysts and mental health program planners

Source: Our World in Data (using IHME/GBD estimates)

Hundreds of Millions Affected: 1 in 3 Women and 1 in 5 Men Face Major Depression

1 in 3

Women experience
major depression
in their lifetime

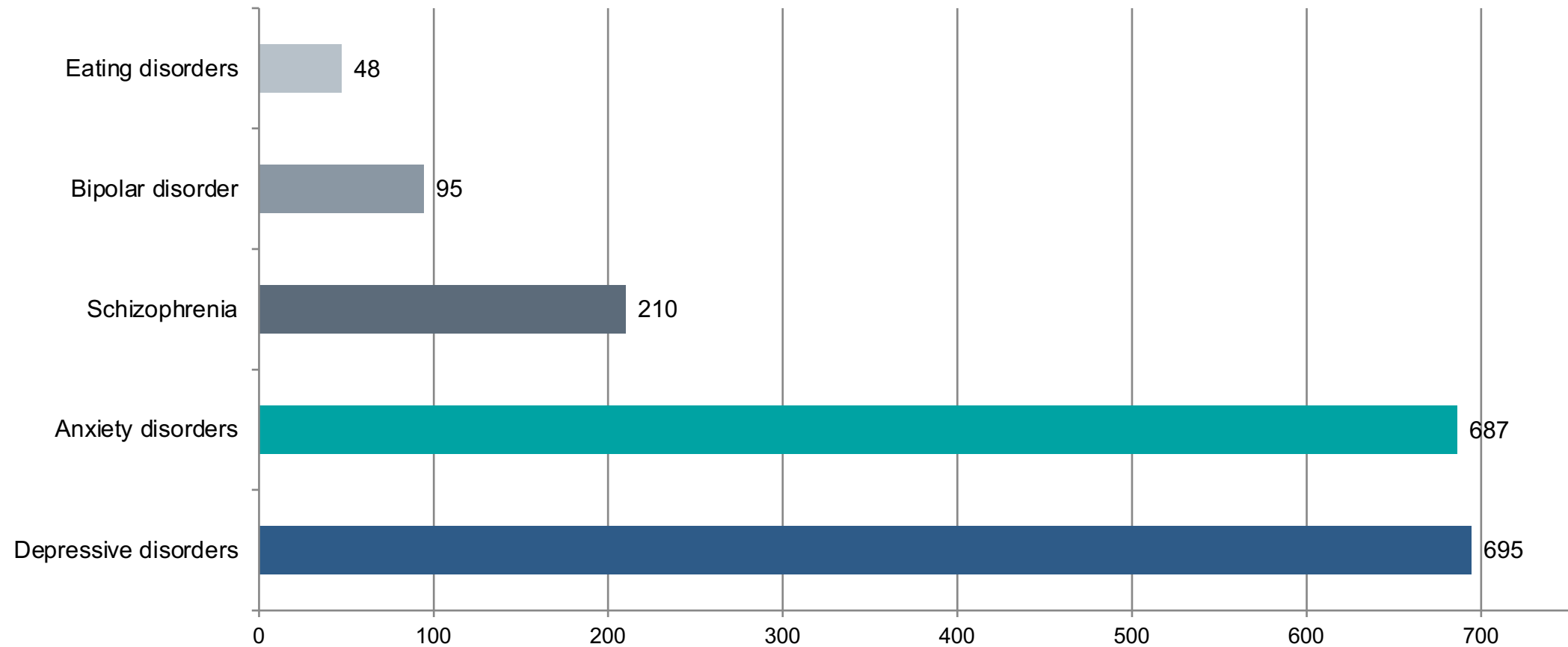
1 in 5

Men experience
major depression
in their lifetime

Mental health conditions span from common disorders (depression, anxiety) to less common but severe illnesses (schizophrenia, bipolar disorder). This scale of need makes mental health a core policy and planning priority.

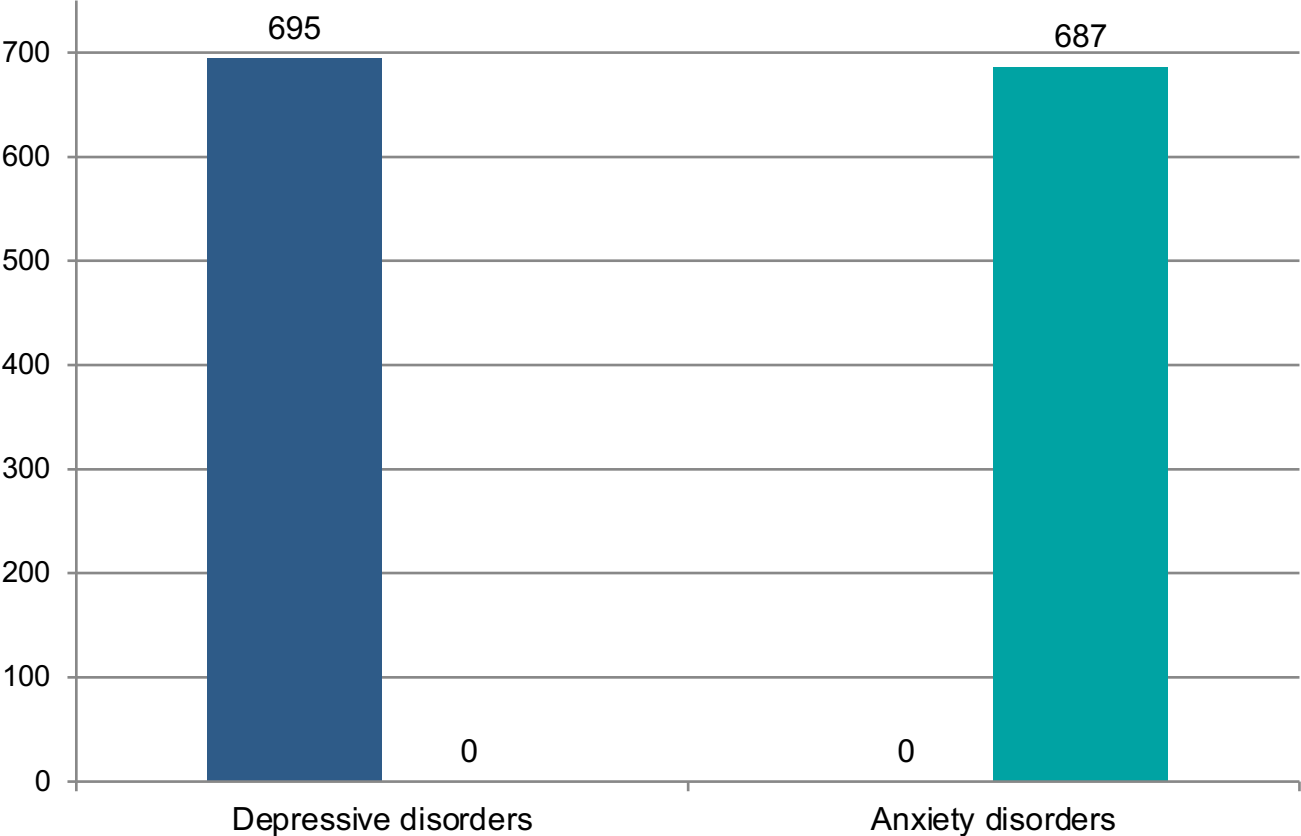
Depression and Anxiety Dominate: ~695 and ~687 DALYs per 100K Respectively

DALYs per 100,000 population by mental illness category (Figure 1)



Source values: depressive disorders 694.6; anxiety disorders 686.5; schizophrenia 210.2; bipolar disorder 94.6; eating disorders 47.5 DALYs per 100,000.

Depression and Anxiety Carry Nearly Equal Disease Burden Worldwide



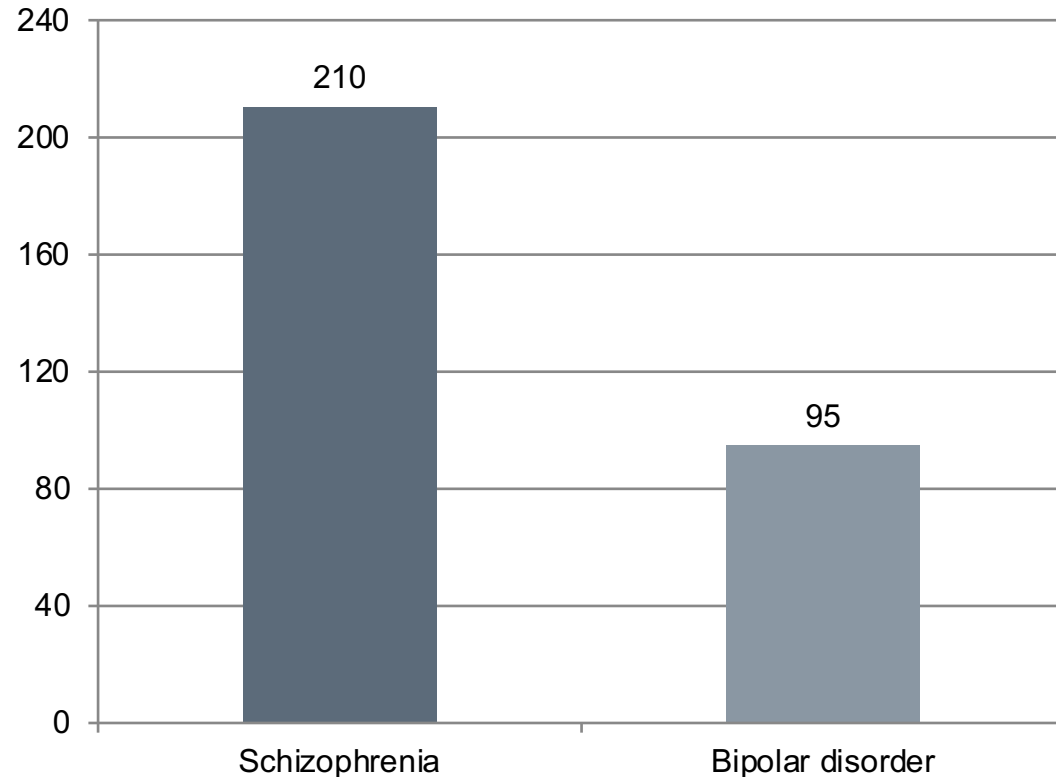
Near Parity

Depression: 694.6
Anxiety: 686.5

Difference: 8.1 DALYs
per 100,000

Together, depression and anxiety account for the vast majority of the mental health burden shown in these DALY estimates, dwarfing other categories.

Schizophrenia and Bipolar Disorder: Lower Prevalence but Severe Individual Impact



Interpretation for policy planners

- Population-level DALYs are lower than depression/anxiety
- Individual-level disability and care needs are often severe
- Service design must preserve specialized care pathways
- Scale common-disorder interventions without neglecting severe conditions

DALYs per 100,000: schizophrenia 210.2; bipolar disorder 94.6

Treatment Is Available but Often Lacking or Poor Quality

1

Stigma

Many people are uncomfortable disclosing symptoms to professionals or people they know.

2

Access

Treatment is often lacking, especially in low- and middle-income countries.

3

Quality

Even where treatment exists, care quality may be poor.

4

Underreporting

Discomfort with disclosure makes true prevalence difficult to estimate.

How People Cope: From Medication to Faith to Family Conversations

Global survey responses indicate substantial cross-country variation in coping behavior (Figure 2).



Prescribed medication

Used by many respondents in some settings; less common in others.



Religious or spiritual activities

Used by many respondents in some settings; less common in others.



Talking to friends or family

Used by many respondents in some settings; less common in others.

Policy implication: design culturally sensitive, context-specific mental health responses rather than one-size-fits-all programs.

Key Takeaways: Prioritizing Depression, Anxiety, and Closing the Treatment Gap

- Depression and anxiety represent the overwhelming majority of mental health DALYs in this comparison.
- Schizophrenia and bipolar disorder show lower population DALYs but high individual burden and disability.
- Treatment exists, yet access and quality remain inadequate in many settings.
- Stigma suppresses help-seeking and contributes to underreporting, obscuring true prevalence.
- Policy should scale evidence-based interventions for common disorders while maintaining specialized care for severe conditions.